TRIAGE POLICY	ONEHEALTH GENERAL PRACTICE & URGENT CARE	
	IND: 2.6.1	UCC STD: 3.2

Associated Documents:

Queue Management
Infectious Disease Management
Workload Policy
Palliative Care Policy
Triage Audit Template
Training and Professional Development

TRIAGE INTRODUCTION.

"Triage" is a French word meaning "sorting, selection, choice." It came from the French verb "trier" meaning "to sort, select, choose" and has been used in English since 1727 to indicate the action of assorting according to quality.

Urgent Care Clinics usually manage lower priority accidents and emergencies than the emergency departments in the public hospitals.

TRIAGE POLICY

OneHealth has adopted a modified Australasian Triage System. The Australasian Triage Score is a system utilised by emergency departments in New Zealand Hospitals. The triage system ensures patient safety and identifies potential emergencies - **Refer Schedule A.**

The OneHealth's Triage Key Performance Indicators (KPI) are as follows:

- 80% of patients triaged within 15 minutes of arrival
- 100% of patients triaged within 25 minutes of arrival

To ensure above KPIs are achieved UCN may need to instigate **Rapid Triage** - **Refer Schedule B.**

NB: Triage can be completed by the Urgent Care Nurse (UCN) or Urgent Care Physician (UCP).

The triage score documented within patient encounter must match the triage score recorded in Queue workflow.

Walkie Talkies are located at reception and are to be used as follows:

1. Off-site Medical Emergency – clinical staff to take a walkie-talkie to ensure they can communicate with reception as and when needed – refer Page 6 for more details.
2. Clinical staff take a walkie-talkie when attending a patient in the red-zone cabin.
   NB: Should a patient be in the Cabin, when no clinical staff are present, a doorbell is available to the patient and when activated, reception staff will escalate to clinical staff that assistance is requried.

Triage Audit

An audit of the Triage process occurs every 3 months, or sooner, should results indicate otherwise. A Triage Audit template is available for this purpose, and a copy is stored within the PPP folder.

75 patients, across the opening hours, are selected for the audit and an incident form is generated for a failed audit.

Triage Associated Signs

- Informing patients of the Triage Process – Located on main waiting room notice board and at front desk.
- Informing patients of Urgent Symptoms - Located next to the fees board in waiting room 1 and on reception desk.

Staff Training

- Non-clinical staff undertake the following:
  - CPR level 2-3 on an annual basis
  - Triage training on commencement of employment and **updated annually** at the September Receptionist meeting.
  - Training on the Paging System, walkie talkies and cabin doorbell is included in the H&S orientation.
- UCPs and UCNs undertake ACLS Level 7 (Core Advanced) training annually.
- GPs and PNs undertake ACLS Level 7 training every 2 years, or sooner.

Self- Triage by Patient via Telephone Call

Should a patient phone the clinic and alert the receptionist to one of the following situations, then the receptionist MUST transfer the call to a clinical staff member:

- Patient is experiencing Chest Pain
- Patient is having an Allergic Reaction
- Patient suspects, or confirms, they have an infectious Disease.
- Patient sounds very depressed, overly anxious, or relays other comments that cause serious concern.

Prior to forwarding the call the receptionist obtains the following information:

- Patient's name
- Patient's Location and Contact Number

The Practice Nurse, or UCN/UCP, are required to manage these calls and on their instruction only, a receptionist can inform the patient to come into the clinic or hang up and call an ambulance. The Clinical staff member must document the telephone call and outcome, in the patient's medical records.

Self -Triage on arrival at the Practice

Each patient presenting to Urgent Care must be screened to ascertain if they are presenting with any Urgent symptoms. To simplify this mandatory step, reception staff can point to the Urgent Symptoms sign, displayed at reception.

URGENT SYMPTOMS

1. Current chest pain or chest discomfort
2. Current shortness of breath
3. Sudden change in vision
4. Severe pain
5. Severe Headache
6. Sudden allergic reaction
7. Severe bleeding from anywhere

8. Poisoning or overdose
9. Suspected Infectious Disease
10. Any other URGENT problem

Patients presenting with Urgent Symptom are categorised as **Triage Alerts**

Suspected Infectious Disease has been included in Urgent Symptoms, to allow for the patient to be isolated and treated in the red zone (Cabin in the top carpark).

A separate notice will be displayed at reception in the event of a pandemic alert. Any patient presenting with associated symptoms are managed as per the process that will be in place at the time of the pandemic. Refer Infectious Disease Management.

For management of Red Zone Triage Alerts, refer SCHEDULE C.

Reception Staff – Triage Guidelines

A receptionist MUST NOT independently give medical advice, or make any clinical decisions, regarding the management of a patient's treatment.

Should patients identify the reason for their visit either verbally, or via a registration form, Receptionist is to record the reason in the queue notes field, as this is extremely helpful for clinical staff.

Reception and nursing staff must be vigilant in ensuring that the estimated wait time is displayed accurately on the electronic fees board. Reception staff should verbally inform patients of the estimated wait times on arrival and make patients aware that it is an estimated time only as presentation of an emergency, or management of a complex patient, may extend the wait time.

This wait time is made in consultation with clinical staff. During periods of increased patient load, or emergencies, it is important reception staff keep patients informed of any changes to the wait time.

It is the responsibility of the on-duty receptionist to ensure the reception desk is not left unattended. If there is only one receptionist on duty, they must liaise with clinical staff prior to leaving Front Desk.

For patient safety, it is important that WR1, WR2 and WR3 are monitored. Cameras have been positioned in each waiting room and can be observed via the viewing monitor located at the Front Desk.

Should a patient request an approximate wait time, prior to being screened for urgent symptoms, the receptionist should state clearly that the estimated wait time is for NON-URGENT patients.

Triage Alerts

Patients are identified, by reception staff, in one of the following ways:

1. Screening at the time of patient's arrival.

2. Registration Forms as they include a section for patients to self-identify Urgent Symptoms.

3. Visually – If patient does not identify with an Urgent Symptom, but patient or support person is distressed, this should be escalated to the UCN for further guidance.

4. Patients who are not identified as a TA, on their initial presentation, but who may develop urgent symptoms whilst waiting for treatment are to be updated to TA on the queue. In this scenario, the TA time **MUST be documented in CONTACTS using the keyword ".TAU"**

Staff MUST NOT override a patient's self-identification of a Triage Alert, even if the patient looks well and does not appear to require immediate attention.

Management of Triage Alerts

When a triage alert is identified the patient **MUST be triaged with 10 minutes of arrival** and the following steps need to be taken:

1. The patient is added to the Queue, and the code 'TA' is selected from the dropdown box in the Notes column.

2. The clinical summary form (CSF) is placed on a red clipboard and placed at the **FRONT** of the To Be Triaged (TBT) tray.

3. When entering the triage notes into Medtech, clinical staff must use the keyword; **".TA"** to reassign the appropriate triage category, ensuring the Queue Triage column is updated with the appropriate Triage Score**. (TA MUST remain in the NOTES column, of the queue, for audit purposes).**

Once the TA patient has been triaged, the UCN replaces the red clipboard with a standard clipboard and places it in the Triaged Tray, in order of Triage Score and within waiting times. UCP is required to follow this order.

Nursing staff - Triage Guidelines

Goals of Triage are:

1. Early identification of the severity of the presenting illness or injury
2. Expedite the patient waiting time and management in the Urgent Care Service

Non-Rapid Triaging Protocol

- Observe patient as they enter the clinic, or the consultation room – clues regarding gait, general presentation, neurological disorders.
- Welcome patients, introduce yourself and briefly explain the triage process.

Obtain a focused history:

- Main complaint
- Duration of main complaint
- Associated symptoms
- Significant past medical and surgical history
- History of allergies
- Current medication

Focused examination:

- Glasgow Coma Scale
- Gait / wheelchair bound.
- Dyspnoea, pallor, diaphoresis, respiratory distress
- Hemorrhage
- Appropriate vital signs / preliminary investigations ie MSU / Visual Acuity

GP Service Hand-over to Urgent Care Service

For continuity of service and safety reasons, a patient booked into the GP service whose condition deteriorates and is assessed with a Triage score of T3 or higher, is handed over to the Urgent Care service.

- GP or PN to hand over immediately to UCP
- Patient is added to the Urgent Care queue.

Paging System

The **PAGING** option is on all desk phones and can be used by all staff to advise of an urgent situation where assistance is required. Training in the paging system is provided within staff's initial Health & Safety training.

MEDICAL EMERGENCIES

Medical Emergencies are those that fall outside of the standard presentation and triage process. These emergencies may require the involvement of all staff on duty or may occur off-site.

The General Practice conducts an Emergency Scenario Drill during annual ACLS / CPR training to ensure all staff are familiar with their roles in a Medical Emergency.

Medical Emergencies in the Waiting Room

Should a medical emergency occur in the waiting room eg a patient has collapsed, or similar and no medical staff are in close proximity, then the receptionist uses the paging system to clearly and calmly announce: **"MEDICAL ASSISTANCE TO RECEPTION".**

Following the announcement, receptionist to assist as follows:

- Proceed to the waiting room and clear the area around the patient to provide good access for medical staff and ensure the surrounding area is safe for patient and staff.
- Be available to assist Medical Staff which may include:
  - Putting patient in recovery position
  - Providing equipment eg Emergency Bag, oxygen, Defibrillator
  - Phoning Ambulance
- Record Patient Registration information, if not yet obtained.
- Provide support to Family / Whanau who have presented with the patient.

Medical Emergencies elsewhere in the Practice

Should an emergency occur in a consultation room, then staff member can either:

- Call out to reception, or other staff, ensuring the request is clear ie help needed from other staff, or call for an ambulance OR.

- The paging system can be used from any room in the clinic that has access to a phone.

Medical Emergencies that occur outside of the OneHealth Practice:

- Only Emergencies within the OneHealth Building and/or perimeter will be attended. The OneHealth perimeter includes:
    - Car Parks at the front of the building and in the basement
    - Remuera Road – immediately in front of the building
    - Beatrice Road - immediately behind the building and accessed via the Exit Door in Resus.
- When notified of an emergency offsite, via telephone:
    - Nurse to undertake brief phone triage/history and assessment of the emergency:
        - ABC's
        - Unconscious or alert
        - Able to move/walk
        - How did the event occur? Accident or Medical Situation
        - Where is the patient? Physical address and location of patient
        - Is there medical equipment available nearby – Defib, Oxygen
    - If the patient is not critical, suggest patient presents to OneHealth.
    - If critical, advise patient or support person, to ring 111 for ambulance. Alternatively, nurse to arrange for staff member to phone 111 on patient's behalf.
- When notified of an emergency offsite, by person physically attending the clinic:
    - Establish location of patient to determine they are within OneHealth's perimeter (If not, ensure 111 has been phoned).
    - Contact person to escort Nurse / Doctor to the site of the medical emergency.
    - Nurse / Doctor to take "emergency bag" and one of the **walkie-talkies** located at reception.
    - Reception staff to check, as clinical team are leaving the practice, that they have the emergency bag and walkie talkie.
    - Reception staff to inform clinic patients, of a possible delay, due to clinical team attending an off-site emergency.
    - **The Emergency Bag MUST be restocked on return.**

For information on other AED's in the area: www.aedlocation

SCHEDULE A

Allocation of triage category

Australasian Triage Category		Treatment Category	Numeric Code
Resuscitation		Immediate	1
Emergency		Within 10 minutes	2
Urgent		Within 30 minutes	3
Semi - urgent		Within 1 hour	4
Non - urgent		Within 2 hours	5

ACUITY ONE:

ATS Category 1 - Immediate simultaneous assessment and treatment

Immediately Life-Threatening Condition

Conditions that are life threatening (or imminent risk of deterioration) and require immediate aggressive intervention.

Clinical Descriptors (indicative only)

- Cardiac arrest
- Respiratory arrest
- Immediate risk to airway - impending arrest
- Respiratory rate <10/min
- Extreme respiratory distress
- BP< 80 (adult) or severely shocked child/infant
- Unresponsive or responds to pain only (GCS < 9)
- Ongoing/prolonged seizure
- IV overdose and unresponsive or hypoventilation
- Severe behavioural disorder with immediate threat of dangerous violence

The medical staff will liaise with the relevant registrar / consultant in the hospital and remain in attendance with the patient until ambulance services and paramedic arrive to expedite the transportation of the patient to hospital

ACUITY TWO:

ATS Category 2 - Treatment within 10 minutes (often simultaneously)

Imminently Life threatening

The patient's condition is serious enough or deteriorating so rapidly that there is the potential of threat to life, or organ system failure, if not treated within ten minutes of arrival **or**

Important time-critical treatment
The potential for time-critical treatment (e.g. thrombolysis, antidote) to make a significant effect on clinical outcome depends on treatment commencing within a few minutes of the patient's arrival in the ED **or**

Very severe pain
Humane practice mandates the relief of very severe pain or distress within 10 minutes

Clinical Descriptors Category 2 (indicative only)

- Airway risk - severe stridor or drooling with distress
- Severe respiratory distress
- Circulatory compromise
- Clammy or mottled skin, poor perfusion
- HR<50 or >150 (adult)
- Hypotension with haemodynamic effects
- Severe blood loss
- Chest pain of likely cardiac nature
- Very severe pain - any cause
- BSL < 2 mmol/l
- Drowsy, decreased responsiveness any cause (GCS< 13)
- Acute hemiparesis/dysphasia
- Fever with signs of lethargy (any age)
- Acid or alkali splash to eye - requiring irrigation
- Major multi trauma (requiring rapid organised team response)
- Severe localised trauma - major fracture, amputation
- High-risk history:
- Significant sedative or other toxic ingestion
- Significant/dangerous envenomation
- Severe pain suggesting PE, AAA or ectopic pregnancy
- Behavioural/Psychiatric:
  - violent or aggressive
  - immediate threat to self or others
  - requires or has required restraint
  - severe agitation or aggression

The medical staff will liaise with the relevant registrar / consultant in the hospital and remain in attendance with the patient until ambulance services and paramedic arrive to expedite the transportation of the patient to hospital.

ACUITY THREE:

ATS Category 3 - Treatment commences within 30 minutes of arrival.

Potentially Life-Threatening
The patient's condition may progress to life or limb threatening, or may lead to significant morbidity, if assessment and treatment are not commenced within thirty minutes of arrival **or**

Situational Urgency
There is potential for adverse outcome if time-critical treatment is not commenced within thirty minutes **or**

Humane practice mandates the relief of severe discomfort or distress within thirty minutes

Clinical Descriptors (indicative only)

- Severe hypertension
- Moderately severe blood loss - any cause
- Moderate shortness of breath
- SAO2 90 - 95%
- BSL >16 mmol/l
- Seizure (now alert)
- Any fever if immunosuppressed eg. oncology patient, steroid Rx
- Persistent vomiting
- Dehydration
- Head injury with short LOC- now alert
- Moderately severe pain - any cause - requiring analgesia
- Chest pain likely non-cardiac and mod severity
- Abdominal pain without high risk features - mod severe or patient age >65 years
- Moderate limb injury - deformity, severe laceration, crush
- Limb - altered sensation, acutely absent pulse
- Trauma - high-risk history with no other high-risk features
- Stable neonate
- Child at risk
- Behavioural/Psychiatric:
    - very distressed, risk of self-harm
    - acutely psychotic or thought disordered
    - situational crisis, deliberate self harm
    - agitated / withdrawn / potentially aggressive

ACUITY FOUR:

ATS Category 4 - Treatment commenced within 60 minutes.

Clinical Descriptors (indicative only)
Mild haemorrhage

- Foreign body aspiration, no respiratory distress
- Chest injury without rib pain or respiratory distress
- Difficulty swallowing, no respiratory distress
- Minor head injury, no loss of consciousness
- Moderate pain, some risk features
- Vomiting or diarrhoea without dehydration
- Eye inflammation or foreign body - normal vision
- Minor limb trauma - sprained ankle, possible fracture, uncomplicated laceration requiring investigation or intervention - Normal vital signs, low/moderate pain
- Tight cast, no neurovascular impairment
- Swollen "hot" joint
- Non-specific abdominal pain

- Behavioural/Psychiatric:
- Semi-urgent mental health problem
- Under observation and/or no immediate risk to self or others

ACUITY FIVE:

ATS Category 5 – Treatment commenced within 120 minutes

Less Urgent
The patient's condition is chronic or minor enough that symptoms or clinical outcome will not be significantly affected if assessment and treatment are delayed up to two hours from arrival, **or**

Clinico-administrative problems
Results review, medical certificates, prescriptions only

Clinical Descriptors (indicative only)
- Minimal pain with no high risk features
- Low-risk history and now asymptomatic
- Minor symptoms of existing stable illness
- Minor symptoms of low-risk conditions
- Minor wounds - small abrasions, minor lacerations (not requiring sutures)
- Scheduled revisit eg wound review, complex dressings
- Immunisation only
- Behavioural/Psychiatric:
    - Known patient with chronic symptoms
    - Social crisis, clinically well patient

Recognition of the Critically Ill Child

Background information
Serious illness in a child may not be recognised. This is because children:
- are poor historians
- may manifest non specific symptoms
- may be uncooperative during examination
- may not show significant indicators - but rather may present as subtle signs
- may be presumed to have age specific diseases

Markers of serious illness in infants under 6 months
High Risk / Medium Risk Five point triage system
1. **Feeding**
   < 1/2 normal
   1/2- 2/3 normal fluid intake
2. **Arousal (CNS)**
   Often drowsy
   Occasionally drowsy
   Drowsiness
   Decreased activity
   Convulsion
   Weak cry

3. **Breathing**
   Apnoea or cyanosis
   Breathing difficulty
   Chest wall recession (indrawing)
4. **Circulation** Skin pale and hot Skin pale Paleness (sudden onset, but persistent)
5. **Fluid output**
   Green vomit > 5 vomits in 24 hr
   <4 wet nappies/ day
   Less urine than usual
   Faeces - Bloody stool

Useful signs

- Alertness drowsiness hypotonic on examination
- Breathing moderate/severe recession cyanosis wheeze
- Circulation pallor signs of dehydration
- Temperature > 38.5C
- Signs of dehydration
- Tender abdomen

Specific signs

- Resp grunt, crepitations, stridor, apnoea tachypnoea >80
- Abdo mass, hernia, distension
- CNS weak cry, abnormal posture
- Skin cold periphery, mottling, bruise, rash
- Pulse > 200
- Urine output < 4 wet nappies

SCHEDULE B: Rapid Triage Process

The purpose of rapid triage is to ensure 80% of patients are triaged within 15 minutes and 100% of patients are triaged, within 25 minutes or sooner, to ensure patient safety.

The key to a successful Rapid Triage process is the communication between UCN, UCP and FD.

Rapid Triage is expected to reduce the triage process to a maximum time of 2 minutes and to achieve this, only the following is obtained and documented:

1. Time triage commenced
2. Quick identification of presenting complaint
3. Relevant Observations and allergies
4. Triage Score

- A TA continues to take priority, during rapid triage, and within 10 minutes of arrival.
  If the TA is a Cardiac Suspected Chest Pain, then an ECG to be initiated at triage (UCN to check with UCP if unsure).

The KPIs below, for After Hours and Normal Hours, are EXCLUSIVE of CHANGE OF DRESSINGS (CODs) AND PLASTER CHECKS (POPC):

AFTER HOURS – WEEKENDS 8am-8pm AND WEEKDAYS 5pm-8pm – Rapid Triage is instigated as follows:

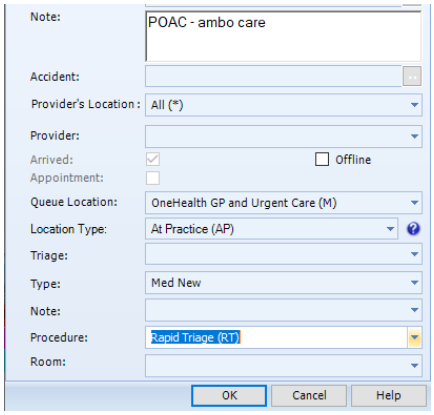
- 1 UCN on duty - When the wait to Triage, **exceeds 15 minutes, with more than 3 patients to triage**.
- 2 UCNs on duty - When the wait to Triage **exceeds 15 minutes, with more than 6 patients to triage.**

NORMAL HOURS – WEEKDAYS 8am–5pm - Rapid Triage is instigated as follows:

- UCN + HCA on duty - When the wait to triage **exceeds 15 minutes, and there are more than 8 patients to triage** (Rapid triage **must** be initiated as OBS will most likely already be done by HCA, and if not, UCN can Rapid Triage whilst the HCA is doing the obs).
- UCN to inform NM and 2IC when Rapid triage is being instigated as they will assist where possible.
- Usually CODs are taken by HCA, but during RAPID triage, obtaining observations of patients to be triaged is the priority.

Process:

- UCN informs FD that Rapid Triage is being instigated.
- FD to update patients in the Queue Arrival column as follows:
  - Patients corresponding CSF is stamped with RAPID TRIAGE.
  - Patient's Queue entry is updated to **capture RAPID TRIAGE in the Procedure field**.
- During Rapid Triage, a patient presenting for a COD or POPC will need to wait until triage is back under control. COD/POPC patients will **NOT** be seen in order of arrival, as they will be left in the awaiting triage column, until UCN is ready to move these patients to the nurse task column. Therefore, FD MUST inform COD & POPC patients that due to the **influx of patients, the UCN has instigated a Rapid Triage process, and some patients will be called ahead of them.**
- When patient is taken for their Rapid triage, either UCN/FD moves patient from Arrival Queue, to In Triage (CTRL E)



- UCN takes the patient through and advises this is a fast triage due to the influx of patients and time taken through is noted on the CSF.
- Relevant observations are taken, check for allergies, document a summary of their presentation, assign a triage score and do **not** initiate investigations.

  NB: UCN can see a few patients to catch-up, then the triage can be documented in the daily records using notes recorded on CSF, as times would have been recorded when patient was moved into the triage column. When documenting in notes, use **RTX keyword:**

  RAPID TRIAGE protocol undertaken due to large influx of patients
  Start time:
  End time:
  Allergies:
  \T \P \BP \WE Resp: Sats:

- When finished, the patient can be returned to the waiting column, or await consultation, depending on their triage score.
- Once the triage waiting time has been reduced, nurses can reinstate the normal triage system and if time allows, reassess triaged patients to initiate investigations, e.g., a urine sample, VA, or X-ray referral, where appropriate.
- When a CSF stamped with Rapid Triage is returned to reception these are stored on a red clipboard until the UCN gives the OK for the forms to be filed with the Daybook CSFs.

AUDIT Requirements

Rapid Triage is included within the Triage audit which is undertaken every three months or sooner if indicated by the audit outcome.

The purpose of monitoring Rapid Triage is to ensure Triage KPIs are being met when the urgent care service has an influx of presenting patients.

SCHEDULE C: Red Zone TA Patients

Patient presenting with a suspected or confirmed infectious disease will be seen in the red zone. However, should they present with another Urgent Symptom, they are identified as a Red Zone TA, whereby the following occurs:

- Receptionist obtains details and location of patient while they await assessment, as they will not be permitted into the practice.
- Patient is loaded on Queue as a Red Zone Triage Alert pt ie **TAC**
- It is crucial that mobile phone numbers are entered correctly in the system.

